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February 23, 2026

The Honorable Robert F. Kennedy Jr.  
Secretary of Health and Human Services  
200 Independence Avenue, SW  
Washington, DC 20201

Submitted via the Federal Rulemaking Web Portal: <http://www.regulations.gov>

**RE: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (HHS Health Sector AI RFI)**

Dear Secretary Kennedy,

The Blue Cross Blue Shield Association (BCBSA) appreciates the opportunity to provide comments on the Department of Health and Human Services (HHS)'s Request for Information regarding accelerating the adoption and use of artificial intelligence (AI) as part of clinical care, as published in the Federal Register on December 23, 2025 (90 FR 60108).

The Blue Cross Blue Shield Association is a national federation of independent, community-based and locally operated Blue Cross and Blue Shield companies (Plans) that collectively provide health care coverage for one in three Americans. BCBS Plans contract with 97% of hospitals and 83% of doctors across the country and serve those who are covered through Medicare, Medicaid, an employer, or purchase coverage on their own.

BCBSA and BCBS Plans share the Administration's commitment to advancing responsible AI innovation that improves patient outcomes, enhances clinical decision-making and reduces administrative burdens—while upholding patient privacy, civil rights and civil liberties. AI technologies are already transforming health care by supporting more efficient care delivery, and improved patient engagement and transparency. AI offers revolutionary solutions to support chronic disease prevention and management through earlier risk detection, timely interventions and better care coordination. At the same time, trust in these tools depends on clear governance and accountability, robust protections for patients, thoughtful transparency, and confidence that AI performs reliably across diverse clinical settings and populations.

We appreciate HHS's emphasis on a coordinated approach across regulation, reimbursement, and research & development to support AI use in clinical care. A predictable, risk-based approach, paired with strong cross-agency coordination, can reduce the uncertainty and complexity that impedes innovation, while ensuring safeguards are commensurate with real-

world risk. As HHS considers how best to use these levers to accelerate adoption, it will be important to prioritize approaches that build public trust, emphasize quality and continued improvement, support real-world implementation and avoid duplicative or conflicting requirements. With that in mind, BCBSA offers the following key recommendations:

- **Harmonize and coordinate federal AI policy, particularly health-specific policies.** HHS should provide strong federal leadership to harmonize AI-related frameworks, guidance and templates with widely accepted standards and existing health care requirements, including alignment across state and federal jurisdictions where feasible. This will reduce regulatory fragmentation, avoid duplicative compliance burdens and provide clearer expectations for organizations developing and deploying AI in clinical care.
- **Adopt flexible, risk-based approaches to policy.** HHS should anchor AI policy in consensus-based, voluntary standards and risk management frameworks that scale governance and accountability to the likelihood and severity of potential consumer harm. A flexible approach avoids one-size-fits-all mandates while ensuring more rigorous safeguards for high-risk clinical use cases.
- **Align reimbursement with demonstrated value.** HHS reimbursement policies should reward clinically valid AI that improves outcomes, safety and administrative efficiency, and should not increase payment solely because AI is used. Tying payment to measurable real-world impact will promote competition based on results and accelerate adoption of AI tools that deliver meaningful patient and system value.

Thank you for the opportunity to provide input on HHS' Request for Information on accelerating the adoption and use of AI as part of clinical care. We welcome continued collaboration with HHS and its Divisions as the Department advances a OneHHS approach that supports responsible AI innovation. If you have questions, please contact Jennifer Jones at 202-942-1269 or [Jennifer.Jones@bcbsa.com](mailto:Jennifer.Jones@bcbsa.com).

Sincerely,



Kris Haltmeyer  
Vice President, Policy Analysis  
Blue Cross Blue Shield Association

## **BCBSA'S DETAILED COMMENTS ON THE REQUEST FOR INFORMATION: ACCELERATING THE ADOPTION AND USE OF ARTIFICIAL INTELLIGENCE AS PART OF CLINICAL CARE**

### **Recommendation #1:**

BCBSA recommends that HHS provide strong federal leadership and coordination to ensure its AI-related regulatory frameworks, implementation activities, guidance documents and any associated templates (e.g., model documentation, attestations, reporting formats, and best-practice playbooks) are harmonized with widely accepted standards and align across federal and state approaches where feasible.

### **Rationale:**

A harmonized approach is essential to reducing uncertainty and accelerating responsible AI adoption in clinical care. Stakeholders are already navigating a rapidly evolving landscape of federal and state initiatives, Executive Orders, and sector-based standards. When requirements and expectations diverge across policies, it creates potentially conflicting obligations and interpretation and implementation challenges for regulators, enforcement entities, developers, deployers and consumers—slowing adoption and increasing compliance costs.

BCBSA generally supports the key definitions in the National Association of Insurance Commissioners (NAIC) AI Model Bulletin, which align well with the National Institute of Standards & Technology (NIST) AI Risk Management Framework (AI RMF), a widely accepted standard. Aligning HHS materials to these common reference points would promote clarity and consistency across the health care ecosystem, enabling organizations to build repeatable governance, risk management and documentation practices that work across programs and settings. Similarly, recognition of health care quality and accreditation efforts, including those emerging from the National Committee for Quality Assurance (NCQA) can help ensure HHS efforts are not duplicative or conflicting.

Harmonization must also account for the unique regulatory environment governing health care and health coverage. Health care differs from many other industries because existing legal frameworks already establish meaningful requirements that directly affect data and AI governance. For example, the Health Insurance Portability and Accountability Act (HIPAA) provides a comprehensive privacy and security framework for HIPAA-regulated covered entities and their business associates. Greater federal coordination and alignment around appropriate, risk-based requirements that build on these existing obligations would help streamline responsible innovation while accelerating the adoption of AI in clinical care. Aligning HHS AI-related materials with established obligations, rather than layering inconsistent or duplicative requirements, will support responsible innovation and reduce the risk of a patchwork of differing state and federal requirements creates duplicative compliance burdens.

### **Recommendation #2:**

BCBSA recommends HHS adopt and promote a flexible, risk-based, standards-driven approach as the foundation for responsible use of AI in clinical care.

### **Rationale:**

Consensus-based, voluntary standards developed by accredited standards organizations grounded in risk-based governance are essential to trustworthy AI. Because AI tools differ

significantly in function, context and potential impact on patients, there is no one-size-fits-all approach to measuring, managing or mitigating risk. Accordingly, HHS policies should scope accountability expectations based on the severity and likelihood of potential consumer harm. Rather than imposing prescriptive requirements, such as mandating specific tests or detailed technical specifications, HHS should leverage and align with flexible, consensus-based risk management frameworks, such as the NIST AI RMF, that allow organizations to tailor risk mitigation to the unique considerations of each AI system. This approach supports innovation while ensuring that safeguards are commensurate with risk, with more rigorous governance, oversight and mitigation measures applied to higher-risk use cases that could materially affect patient safety and clinical outcomes.

### **Recommendation #3:**

BCBSA recommends that HHS establish a structured, ongoing stakeholder engagement process to inform HHS AI policy development and implementation.

#### **Rationale:**

Participation should include a diverse group of stakeholders, including consumers and consumer groups, technical experts and private-sector entities that will be directly and indirectly impacted—including health insurers. AI policy is being developed in a rapidly changing environment where the knowledge base is evolving quickly and where there is still limited real-world experience regulating and operationalizing AI in clinical settings. A broad, diverse cross-section of stakeholders is essential to ensure HHS approaches are workable across different care settings and to avoid requirements that inadvertently create barriers to adoption or impose burdens that do not meaningfully improve outcomes or protections. Because AI is highly use-case specific, “real-world” input is critical to designing requirements that can be implemented consistently and at scale.

### **Recommendation #4:**

BCBSA recommends that HHS support education and workforce readiness initiatives to facilitate responsible implementation of AI in clinical care.

#### **Rationale:**

Successful adoption of AI in clinical care depends not only on the technology itself, but also on the readiness of the workforce and the availability of practical implementation support. Many organizations face challenges integrating AI into real-world clinical workflows, establishing appropriate governance, and ensuring staff understand how to use AI tools safely and effectively. Targeted education and workforce readiness efforts can help clinicians, administrators and other users develop the competencies needed to evaluate AI outputs, apply appropriate human oversight and respond to limitations or performance issues. HHS can also reduce uncertainty by advancing additional study and implementation-focused research that identifies effective practices across settings and populations, including smaller and rural providers.

### **Recommendation #5:**

BCBSA recommends that HHS focus AI-related public policy and any resulting requirements to artificial intelligence systems (AIS), rather than broadly regulating “algorithms.”

**Rationale:**

Algorithms can describe an extremely broad set of processes, including simple business rules and statistically driven empirical approaches that do not necessarily involve AI or advanced computing. If HHS policies are written to cover “algorithms” broadly, they risk sweeping in commonplace operational logic and established analytical methods that do not present the same governance, transparency or safety issues as AIS used in clinical care. The focus should remain on the functional autonomy and emergent risk of the system rather than its underlying algorithmic components. An overly broad application can drive inconsistent interpretation across regulators and increase compliance costs without improving consumer protections. This can also dilute oversight by spreading attention across low-risk, non-AI tools, rather than focusing on AIS uses that may meaningfully affect clinical decisions, patient experiences, and outcomes. A clear AIS-focused approach better supports HHS’s goals of predictability and proportionality, it enables HHS to tailor expectations to risk, strengthen trust where it matters most and reduce uncertainty that can impede innovation and adoption.

**Recommendation #6:**

BCBSA recommends that HHS support and operationalize systematic, continuous real-world monitoring of medical devices that use AI through aligned expectations, practical infrastructure and incentives that promote routine post-deployment performance evaluation and transparency over time.

**Rationale:**

AI tools used in clinical care must perform safely and effectively in the diverse real-world settings and patient populations where they are deployed, not only in pre-deployment testing environments. Yet, oversight today remains centered on premarket clearance and limited post market surveillance, providing only snapshots of performance at a single point in time. This gap is particularly important for payers, clinicians and patients, because coverage, quality and care decisions depend on longitudinal evidence that the tool continues to perform as intended over time.

Continuous post-deployment monitoring is especially important for medical devices that use AI because performance can change as clinical workflows evolve, input data shift, vendor updates occur or patient demographics change. Without systematic monitoring, it is difficult to identify and address model drift, strategic drift (i.e., when an AIS gradually stops reflecting an organization’s clinical policies and ethical guardrails over time, even if accuracy metrics appear stable), degraded accuracy or emergent safety issues. This can undermine public trust and create uncertainty for organizations considering adoption, precisely the uncertainty HHS seeks to reduce. HHS is uniquely positioned to accelerate real-world monitoring by encouraging common expectations and reusable approaches across the Department’s policy levers. More consistent real-world monitoring would also help promote market competition based on demonstrated performance and value and improve confidence in AI-enabled care.

**Recommendation #7:**

BCBSA recommends that HHS advance a risk-based, lifecycle approach to testing, evaluation, validation, and verification (TEVV) for AI used in clinical care, coordinated with NIST and other standards-setting organizations, so that TEVV expectations are practical, consistent and proportionate to an AI tool’s clinical impact, intended use, and potential risk.

**Rationale:**

A lifecycle TEVV approach is essential because AI tools are not “set-and-forget” technologies. A risk-based lifecycle TEVV approach, covering both pre-deployment evaluation and ongoing reassessment after deployment, helps ensure safeguards are commensurate with real-world risk. This proportionate approach supports public trust while avoiding burdens that could slow adoption of lower-risk tools. Coordination with NIST and other standards-setting organizations can help establish consensus frameworks for real-world AI evaluation that prioritize transparency and risk alignment. This coordination should build on NIST’s foundational AI RMF and identify opportunities for these frameworks to be meaningfully leveraged across the AI lifecycle in clinical settings. Alignment around widely accepted frameworks is key to minimizing compliance friction. Lifecycle TEVV supports HHS’s reimbursement and R&D objectives by strengthening the evidence base on benefits, costs and tradeoffs associated with AI in clinical care. When TEVV is tied to real-world evaluation and implemented proportionately to risk, it can help ensure that adoption is driven by demonstrated value and that patients are protected.

**Recommendation #8:**

BCBSA recommends HHS structure AI clinical care reimbursement policies to promote adoption of clinically valid AI tools that demonstrate improved patient outcomes, patient safety and care team efficiency. Reimbursement should support adoption and scaling of AI that measurably reduces administrative burden and improves the delivery of care, not simply the additional presence of AI.

**Rationale:**

AI is increasingly embedded in clinical care, but not all AI-enabled tools deliver meaningful improvements in outcomes or efficiency. Payment policies that provide additional reimbursement simply because AI is used risk encouraging adoption based on tool novelty rather than patient value and can create unintended incentives that increase spending without commensurate benefit. Aligning reimbursement with existing expectations and principles for other clinical interventions, such as demonstrated clinical validity and measurable real-world impact helps ensure patients benefit from AI that supports safer decision-making, improves quality of care and reduces delays in appropriate care. Reimbursement policy should also recognize value where AI streamlines clinical and administrative workflows in ways that improve provider experience, including by reducing the “context tax,” or the time and resource burden clinicians face synthesizing fragmented patient information across systems. Structuring reimbursement around demonstrated value and patient-centered outcomes can help accelerate adoption of AI tools that improve care and affordability.

**Recommendation #9:**

BCBSA recommends HHS encourage adoption of high-value AI through value-based payment models and performance-based approaches that reward measurable improvements in quality, efficiency, and patient outcomes.

**Rationale:**

Value-based payment models are well suited to promote AI adoption because they align incentives around outcomes and efficiency rather than volume. When providers are accountable for quality and total cost of care, they have stronger incentives to deploy AI tools that improve

care coordination, reduce avoidable utilization and streamline workflows in ways that benefit patients. Incorporating AI-related performance expectations into value-based benchmarks can also promote competition among developers based on real-world impact, rather than creating standalone reimbursement that rewards AI use regardless of benefit. This approach helps ensure that adoption is driven by tools that deliver measurable improvements for patients and the health care system, while reducing the risk of spending increases tied to add-on technologies that do not improve outcomes.